

 **My strongest alternative: Rural Healthcare AI**

□ “JEEVANSETU AI”

AI-powered Rural Health Screening, Referral & Emergency Support Platform

Theme: MedTech / BioTech / HealthTech

Target: Rural and underserved communities

There is an actual SIH 2026 problem statement, **SIH26133**, focused on improving the accessibility and quality of public healthcare services, particularly in rural and underserved areas.

This is the kind of project I would seriously consider for your team.

Why this can have BIG impact

Think about a person in a remote village.

They may have:

- no nearby specialist
- limited diagnostic facilities
- long travel distance to hospitals
- difficulty explaining symptoms
- delayed referrals

Your system becomes a **first-level AI health screening and referral assistant**.

But ⚠

Don't present it as:

✗ "AI diagnoses diseases."

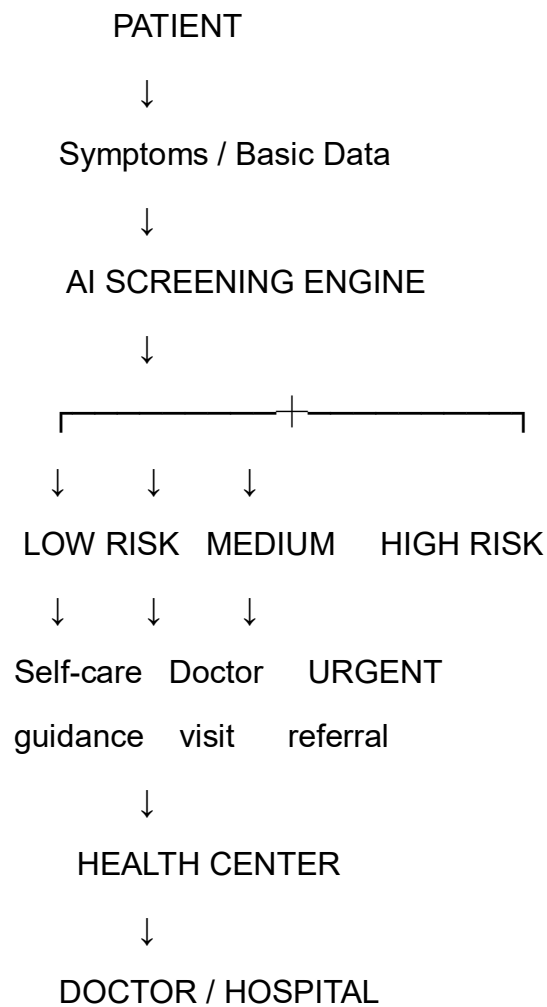
That's risky and medically inappropriate.

Present it as:

✓ **"AI-assisted preliminary risk screening and referral support."**

The final medical decision remains with a qualified healthcare professional.

HOW YOUR SYSTEM WORKS



But here's where you make it **much more powerful**.

FEATURE 1 — AI Risk Screening

User enters:

Fever

Cough

Difficulty breathing

Age: 62

Oxygen: 91%

System produces:

● **HIGH PRIORITY**

Reasoning:

- Low oxygen saturation
- Breathing difficulty
- Age-related risk

Recommendation:

Seek urgent medical evaluation.

It should **not** claim:

"You have pneumonia."

FEATURE 2 — Rural Health Triage

This could be your biggest USP.

Instead of simply predicting a condition, classify:

☐ **ROUTINE**

Visit healthcare center when convenient.

☐ **PRIORITY**

Consult healthcare worker soon.

☒ **URGENT**

Seek immediate medical attention.

This makes the system useful even when exact diagnosis isn't possible.

FEATURE 3 — Nearest Appropriate Healthcare Center

This is where GIS comes in.

Suppose:

Patient location

↓

AI identifies:

High-priority case

↓

System finds:

 nearest PHC

 nearest CHC

 nearest government hospital

and shows:

Nearest PHC

Distance: 4.8 km

Estimated travel time: 14 min

Services:

- ☐ General consultation
- ☐ Basic laboratory
- ☐ Emergency stabilization

FEATURE 4 — Hospital Capacity Awareness

This is where I would make your project different from ordinary healthcare apps.

Suppose:

Facility Distance Emergency Beds

PHC A	4 km	Limited	2
CHC B	8 km	Yes	12
Hospital C	18 km	Yes	46

AI recommends:

 **CHC B**

because it has the required emergency capability.

So you're not merely saying:

"Nearest hospital."

You're saying:

"Nearest suitable healthcare facility."

FEATURE 5 — Multilingual Voice Assistant IN

This could make the demo AMAZING.

User speaks in Telugu:

"నాకు మూడు రోజులుగా జ్వరం ఉంది..."

System:

 Speech → Text

↓

 AI screening

↓

□ Risk assessment

↓

 Response in Telugu

For rural accessibility, **voice-first interaction** can be more meaningful than a text-heavy interface.

FEATURE 6 — Offline Mode

This is a **killer feature**.

Many rural locations can have unreliable connectivity.

Your mobile application stores:

- basic screening logic
- emergency instructions
- nearby healthcare facilities
- previous patient records locally

When internet becomes available:

OFFLINE

↓

Screen Patient



Store Data



NETWORK AVAILABLE



SYNC



Doctor Dashboard

FEATURE 7 — Doctor Dashboard

Doctor sees:

Today's Cases

 7 urgent

☐ 14 priority

☐ 28 routine

Click patient:

Patient ID: P1024

Age: 62

Symptoms:

☐ Fever

☐ Cough

☐ Breathlessness

SpO₂: 91%

Risk: ● HIGH

AI reasoning:

Low SpO₂ + respiratory symptoms

Action:

URGENT CLINICAL EVALUATION

FEATURE 8 — Patient Health Timeline

Instead of treating every visit independently:

JAN

Fever



MAR

Breathing issue



JUN

High BP



SEP

Current symptoms

The doctor gets a **longitudinal view**.

FEATURE 9 — Health Worker Mode

This could be your biggest real-world feature.

Imagine an **ASHA/ANM/community health worker** visiting a village.

They open the app.

SELECT PATIENT

↓

Collect:

- age
- symptoms
- BP
- SpO₂
- temperature
- blood glucose
- heart rate

↓

AI gives:

PRIORITY LEVEL

↓

Worker knows:

"This patient needs referral."

This creates a bridge:

Village → Health Worker → PHC → Hospital



FEATURE 10 — Population Health Dashboard

For administrators:

VILLAGE HEALTH STATUS

Respiratory complaints  18%

Fever  14%

Diabetes risk  11%

Hypertension  9%

If one village suddenly reports many similar symptoms:



ANOMALY DETECTED

Unusual increase in respiratory complaints.

This can help authorities investigate emerging local health issues.

□ YOUR AI ARCHITECTURE

Patient Data



Data Validation



Feature Engineering



Risk Models



Explainable AI



Triage Classification



Healthcare Facility Matching



Doctor / Health Worker



Patient

Technologies

Frontend

- React.js
- Tailwind CSS
- Leaflet

Mobile

- Flutter

Backend

- Python
- FastAPI

AI

- Scikit-learn
- XGBoost
- SHAP

Database

- PostgreSQL
- PostGIS

Voice

- Speech-to-text
- Text-to-speech

Authentication

- Firebase/Auth

WHAT MAKES IT DIFFERENT?

Don't build another:

"AI symptom checker."

There are already countless versions of those.

Build:

**AI + TRIAGE + HEALTHCARE ACCESS + GIS + VOICE + OFFLINE + DOCTOR
CONNECT**

That's much stronger.

IMPACT STORY

Your PPT should show this:

TODAY

Village



Symptoms



Travel 20–50 km



Hospital



Diagnosis

WITH JEEVANSETU

Village



Health Worker



AI Screening



Risk Classification



Suitable Healthcare Center



Doctor

Result:

Earlier identification → faster referral → better access ..